# Sampson Physical Therapy

585 Raymond Field
Pasadena, TX
Phone 346-555-1390

## Itemized Medical Bill

| | |
|---|---|
| **Patient:** | Kathleen Gamble |
| **Date of birth:** | 04/04/1952 |
| **Insurance claim:** | C-1386 |
| **Date of injury:** | 01/10/2025 |
| **First visit:** | 01/11/2025 |
| **Last visit:** | 05/05/2025 |
| **Number of visits:** | 19 |

### Services

| Service | Units | Charge |
|---|---|---|
| 99203 New patient evaluation | 1 | $259.51 |
| 98941 Spinal manipulation | 3 | $367.62 |
| 97110 Therapeutic exercise | 5 | $774.82 |
| 97124 Massage therapy | 5 | $515.69 |
| 72040 X-ray, cervical spine | 2 | $1,125.13 |
| 20552 Trigger point injection | 3 | $1,544.76 |
| TOTAL | | $4,587.53 |

### Recent visits

| Date | Service | Charge |
|---|---|---|
| 04/08/2025 | 97124 Massage therapy | $102.17 |
| 04/13/2025 | 98941 Spinal manipulation | $118.51 |
| 04/22/2025 | 97124 Massage therapy | $88.50 |
| 04/26/2025 | 98941 Spinal manipulation | $113.51 |
| 04/30/2025 | 97124 Massage therapy | $100.45 |
| 05/05/2025 | 97124 Massage therapy | $107.21 |
| 05/05/2025 | 97112 Neuromuscular re-education | $127.54 |

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## Treatment History and Summary

| | |
|---|---|
| **Patient:** | Kathleen Gamble |
| **Treating provider:** | David Dickerson, DC |
| **Diagnosis:** | knee injury |

### Treatment history by month

| Month and services | Visits | Charges |
|---|---|---|
| January 2025: chiropractic adjustment, evaluation, physical therapy | 3 | $521.79 |
| February 2025: imaging, pain management, physical therapy | 4 | $1,123.53 |
| March 2025: massage therapy, pain management, physical therapy | 5 | $1,617.86 |
| April 2025: chiropractic adjustment, imaging, massage therapy | 6 | $1,217.14 |
| May 2025: massage therapy | 1 | $107.21 |

### Treatment summary

The patient presented following a rear-end mootr vehcile collision with complaints of right knee pain and reduced range of motion. The reported mechanism of injury is consistent with the knee injury described, and the presentation is plausible for a collision of this type. Over the course of 19 visits, care included physical examination, imaging review, a structured rehabilitation program, and activity modification. The treatment course followed a typical progression, with visit frequency decreasing and symptoms easing as the patient improved. Some examination findings, including joint line tenderness and reduced flexion, support the reported injury, while other orthopedic tests and neurovascular checks were within normal limits. No prior injury to the same knee is known from the patient history. Billing records show two similar charges on the same date of service, which are currently under review for accuracy and duplication. Ongoing monitoring and a home exercise plan were recommended at discharge from active care.